

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted		☒
Case Is (Circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	RB	Date Reviewed
		4/8/11

Specimen Description

105-0-3
 carcinoma, papillary + follicular 8340/3
 Site: thyroid, NOS C73.9 lw 4/8/11

Surgical Pathology Report

*****Clinical History*****

History: Right thyroid mass.

*****Final Pathologic Diagnosis*****

- A. Thyroid lobe, right, lobectomy:
 - Combined follicular carcinoma and papillary thyroid carcinoma
 (see synoptic report).

Note: the lesion is composed predominantly of a follicular proliferation showing extensive capsular invasion, with penetration of tumor through the capsule. The nuclear features of the follicular carcinoma component are those of conventional follicular cells and these areas do not display the nuclear or architectural features of papillary carcinoma. In other areas of the nodule, however, the tumor shows well-developed nuclear features of papillary carcinoma, with the formation of papillary structures and abundant psammoma bodies. The tumor is seen to infiltrate into the perithyroidal soft tissue and reaches up to the inked outer surface of the specimen. A small perithyroidal lymph node attached to the edge of the isthmus margin is involved with metastatic papillary carcinoma. Tumors showing combined features of papillary and follicular carcinoma are extremely rare and most often correspond to the follicular variant of papillary carcinoma. The nuclear and architectural features of the follicular component in this lesion, however, are not those of the follicular variant of papillary carcinoma. This case thus appears to represent an example of a true mixed, or combined follicular/papillary carcinoma of the thyroid.

Electronically Signed By

*****Synoptic Report*****

TCGA

Please note: This was re-reviewed
 by on 3/21/11 and
 classified as a PTC, follicular
 variant.

UUID: C66683CA-DB7D-4451-AC28-E91FD6FA651E
 TCGA-FE-A237-01A-PR

Redacted



 Print this Page**Specimen Description****Surgical Pathology Report**

Patient Name: [REDACTED]
Accession #: [REDACTED]
Med. Rec #: [REDACTED]
Submitting Physician: [REDACTED]

*****Clinical History*****
History: Right thyroid mass.

*******Final Pathologic Diagnosis*******

A. Thyroid lobe, right, lobectomy:
- Combined follicular carcinoma and papillary thyroid carcinoma
(see synoptic report).

Note: the lesion is composed predominantly of a follicular proliferation showing extensive capsular invasion, with penetration of tumor through the capsule. The nuclear features of the follicular carcinoma component are those of conventional follicular cells and these areas do not display the nuclear or architectural features of papillary carcinoma. In other areas of the nodule, however, the tumor shows well-developed nuclear features of papillary carcinoma, with the formation of papillary structures and abundant psammoma bodies. The tumor is seen to infiltrate into the perithyroidal soft tissue and reaches up to the inked outer surface of the specimen. A small perithyroidal lymph node attached to the edge of the isthmus margin is involved with metastatic papillary carcinoma. Tumors showing combined features of papillary and follicular carcinoma are extremely rare and most often correspond to the follicular variant of papillary carcinoma. The nuclear and architectural features of the follicular component in this lesion, however, are not those of the follicular variant of papillary carcinoma. This case thus appears to represent an example of a true mixed, or combined follicular/papillary carcinoma of the thyroid.

Electronically Signed By
[REDACTED]
[REDACTED]

*******Synoptic Report*******

SYNOPTIC REPORT FOR THYROID CANCER:
Specimen type: Lobectomy.
Tumor location: Right lobe.

SYNOPTIC REPORT FOR THYROID CANCER:

Specimen type: Lobectomy.

Tumor location: Right lobe.

Tumor size: 3.5 x 3.0 x 3.0 cm.

Histologic type: Combined follicular and papillary thyroid carcinoma.

Tumor grade: Well-differentiated.

Lymphatic/vascular invasion: No.

Capsular invasion: Yes, extensive.

Tumor extent:

Confined to the thyroid: yes.

Multifocal: No.

Invasion of thyroid cartilage or hyoid bone: No.

Infiltration of adjacent structures: Perithyroidal soft tissue.

Margins:

For partial or subtotal resection: Isthmic margin free of tumor.

Lymph nodes:

Total number examined: 1

Sites/laterality: Right.

Number positive: 1

Number negative: 0

Extranodal extension: No.

Additional pathologic findings: None.

pTNM: pT2N1Mx.

The above synoptic report complies, in slightly modified form, with the guidelines of the College of American Pathologists and the Association of Directors of Anatomic and Surgical Pathology for the reporting of cancer specimens

*****INTRAOPERATIVE CONSULTATION DIAGNOSIS:*****

- AF1. Right thyroid lobe: (FROZEN SECTION PERFORMED)
- Follicular lesion, favor benign.
- Defer to permanent for final diagnosis.

Examining pathologist: [REDACTED]

*****SPECIMEN(S) RECEIVED:*****

Thyroid, REG

*****GROSS DESCRIPTION:*****

The specimen is received in one properly labeled container with the patient's name and accession number.

A. The specimen is designated "right thyroid lobe" and consists of a 14.6 gram lobe of thyroid that measures 6.2 x 3.0 x 2.5 cm. The external surface is tan-pink and smooth with no lymph nodes or parathyroid gland identified. The external surface of the thyroid is inked black and the isthmic margin is inked blue. Cut sections reveal a 3.5 x 3.0 x 3.0 cm

tan, irregular, unencapsulated nodule in the inferior aspect of the right lobe that focally abuts the capsule and comes to within 1.3 cm of the isthmic margin. The nodule is tan and homogeneous with no areas of hemorrhage or necrosis. The nodule does not appear to display gross infiltration of the capsule. The surrounding uninvolved thyroid parenchyma is deep red and displays no additional lesions. The entire nodule is submitted for microscopic examination. A random section of the nodule is submitted for frozen section and resubmitted as received in cassettes AF1.
(P) RS 13

Summary of Cassettes: AF1, two representative sections of nodule; A1, isthmic margin, shave; A2-11, entire nodule submitted from lateral to medial

Gross description by: [REDACTED]